



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : امل رياض عبد الخالق حسين

العنوان : قصر الباسل مركز
اطلسا الفيوم

الحي / المدينة : الشيخ زايد

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 28-09-2026

رقم البطاقة : 28901242300401 رقم التليفون : 01099544602

العمر (تاريخ الميلاد) : 24-01-1989 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك الإخصائي الاجتماعي

المقر بما فيه التوقيع: (-----)

التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان

الوظيفة: -----
الوظيفة: -----
الوظيفة: -----

التوقيع: -----
التوقيع: -----
التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----

قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: امل رياض عبد الخالق حسن National ID: 28901242300401	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

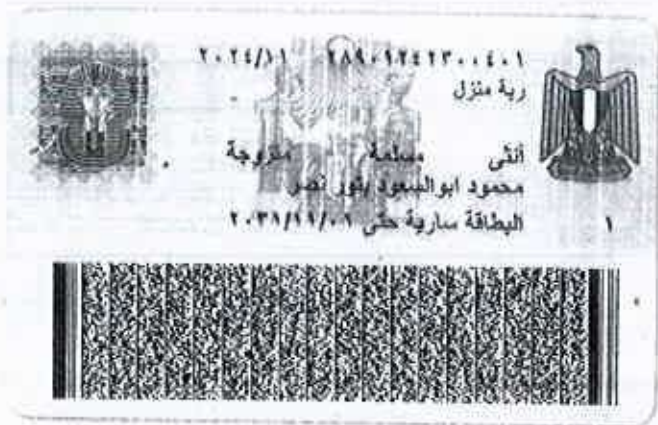
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM

7/9/20

G

Name	امل, راجح فيال	MRN	7509
Age	36	ID	
Menopausal status	premen	Phone	019906719
NRS SCORE		Marital status	CA90630000
BMI		Family history	
CO morbidity	-ve	REASON OF MDT	

BX: IDc Clinical Data

ER: 518 PR: 018 HER 2: 2uph Ki67:

Sonomammography: Denar met Bc
to liver. Bm, Bm

Mets Workup: Pt start Taxol, Herceptin
pt received 36 wk of taxol &
14 cycles of Herceptin
Pt develop PAIN GUT

Recommendations:

Last CT cap 4-2-25 → D.R.
MRI b/w 2-6-25 → Sol's ab 3 to 14mm

MDT OUTCOME & PLAN

Jo Maintenance AI ± 2 levels of hCG level
+ Monitor Herceptin IV

SIGNATURE

Medical Oncology	Surgery	Radiology	Pathology	Nutrition	Psychiatric



Patient Name : الأستاذة / أمل رياض عبد الخالق حسين	Request No. : 13009702
Patient No. : 28901242300401	Request Date : 07/08/2024
Path No. : SR-1099/24	Verification Date : 20/08/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 35 Y	Department : Out Patient
Referring Doc :	Registered by : nianwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Slide revision.

Gross Examination:

- Five paraffin blocks labeled by (840536) and Nine slides were received.

Microscopic Examination:

- * Histological type: Multifocal invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
- * DCIS: Positive (90%) (comedo, cribriform, solid)
- * Angioinvasion: No.
- * Perineural invasion: No.
- * IPT:
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: No nuclear reaction in the tumor cells (score 0/8).
- Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).

Diagnosis:

Right breast cancer, Slide revision:
Multifocal invasive duct carcinoma, Grade II with DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+).
- For further IPT (SISH).

Signature

Verified by :

Dr. Sagda El-Kady

Printing Date : 17/12/2024 10:38:06

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	: الأستاذ / أمل رياض عبد الخالق حسين	Request No.	: 13009702
Patient No.	: 28901242300401	Request Date	: 07/08/2024
Path No.	: SR-1099/24	Verification Date	: 20/08/2024
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 35 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Slide revision.

Gross Examination:

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Microscopic Examination:

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- For further IPT (SISH).

Signature

Verified by :

Dr. Sagda El-Kady



Patient Name: Amaal Reyad Abd Al Khalek Hussein	Date : 10/05/2025
Exam : C.T CHEST WITH CONTRAST	Age : 36 Year(s)
Medical ID: 2509/2024	Pat. ID: 77262

POST -CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

➤ Known case of right BCS . **As compared to the last study 23-01-2025, The current revealed:

1. Chest:

- Right breast still shows rather stationary appearance of the UIQ ill-defined parenchymal asymmetry with diffuse skin thickening
- **Stable course** regarding to the size of the right axillary level I, II and III enlarged lymph nodes became prominent the largest measures about 1x0.5 cm with irregular outlines and ill-defined axillary sheets.
- Non appreciated forenoted Prominent right internal mammary lymph nodes.
- **Stationary** course regarding to size of Few bilateral scattered tiny mostly subpleural / pleural based pulmonary nodules, the largest is noted at inferior lingular segment still measuring 4 mm.
- **Still noted unchanged** bilateral apical pleuropulmonary scarring.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is of average size with **rather stationary course regarding number and size of** bilobar discrete and confluent hypodense focal lesions, The largest at hepatic dome measuring currently about 1.3 x1 cm.
- Patent homogenous portal vein and its main tributaries
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.

Patient Name: Amaal Reyad Abd Al Khalek Hussein	Date : 22/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 36 Year(s)
Medical ID:	Pat. ID: 77262

- Normal size & parenchymal texture of the spleen with no focal lesions seen.
 - No pathological abdominal or pelvic lymph nodes.
 - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
 - Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
 - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
 - Currently noted bilateral adnexal cystic lesions for sonographic assessment.
 - Normal appearance of the other pelvic organs.
 - No abdominal or pelvic masses or collections.
3. **The scanned skeleton:**
- Sacralized L5 vertebra. Stationary course of D4, L1 vertebral body and left iliac wing lytic lesions measuring 7 mm. Small D5 sclerotic lesion. 1st degree lytic spondylolisthesis of L4.
 - No vertebral structural collapse or extra-osseous soft tissue component.
- OPINION:**
- Reported case of right BCS SHOWING***
- Overall Regressive course since the last study.

Dr/Aya Gabr

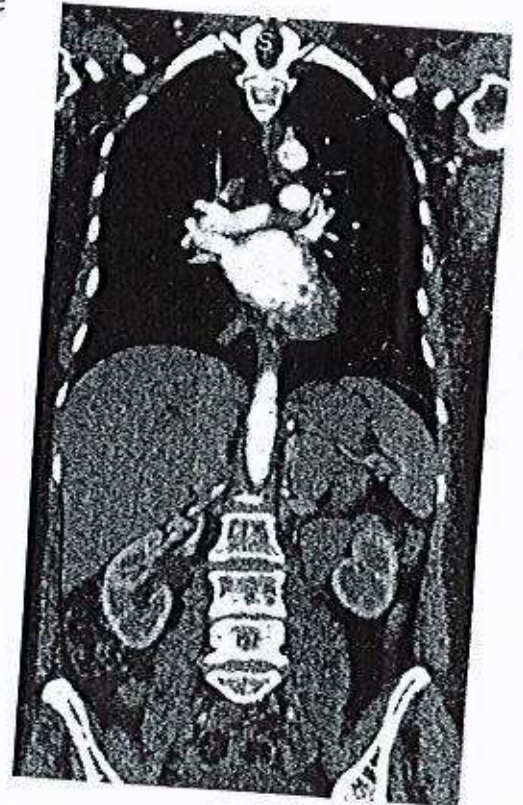
Patient Name: Amaal Reyad Abd Al Khalek Hussein	Date : 22/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 36 Year(s)
Medical ID:	Pat. ID: 77262

POST -CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

➤ **Known case of right BCS . **As compared to the last study 10-05-2025, The current revealed:**

1. Chest:

- Right breast still shows rather stationary appearance of the UIQ ill-defined parenchymal asymmetry with diffuse skin thickening
- **Regressive course** regarding to the size of the right axillary level I, II and III enlarged lymph nodes became prominent the largest measures about 0.5x0.3 cm with irregular outlines and ill-defined axillary sheets.
- Non appreciated forenoted Prominent right internal mammary lymph nodes.
- **Regressive** course regarding to size of Few bilateral scattered tiny mostly subpleural / pleural based pulmonary nodules, seen less appreciated in the current study.
- **Still noted unchanged** bilateral apical pleuropulmonary scarring.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.



2. Abdomen and pelvis:

- The liver is of average size with **Regressive course regarding number and size of** bilobar discrete hypodense focal lesions, seen less appreciated in the current study
- Patent homogenous portal vein and its main tributaries
- No intra-hepatic or extra-hepatic biliary radicles dilatation.

Patient Name: Amal Riad Abd El-Khalek Hussein
Prof. Dr: Clinics Masr El-Hadetha
Accession: 240903134

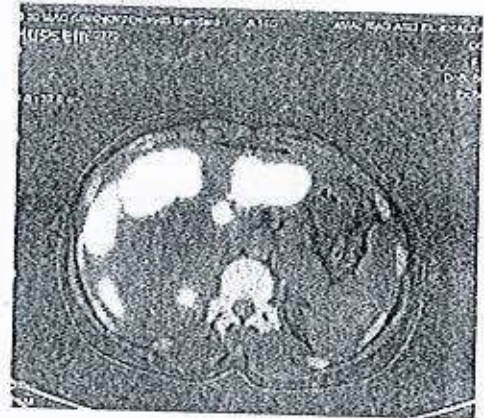
Patient ID: 684364
Exam Date: 3/9/2024

III) Chest:-

- No metabolically active (FDG avid) pulmonary nodules or masses are noted.
- No metabolically active (FDG avid) pleural effusion is noted.
- No FDG avid (metabolically active) mediastinal lymph-nodes are noted.
- Normal cardiac size with no pericardial effusion.

IV) Abdomen and pelvis:-

- Mildly enlarged liver showing few metabolically active ill-defined bilobar hypodense hepatic focal lesions measuring up to 7 x 5.6 cm with max SUV ~ 12.77 (at hepatic dome lesion).
- Metabolically active few enlarged aortocaval, portocaval and porta hepatis lymph nodes are noted measuring up to 3 cm with max SUV ~ 10.45 (at porta hepatis hepatic lymph node).
- Average sized spleen showing no metabolically active (FDG avid) focal lesions.
- No ascites.
- Metabolically active right adnexal cyst is noted measuring 2.7 cm with max SUV ~ 7.18 (corpus luteum).
- The rest of the abdominal and pelvic organs show normal PET/CT appearance with no related focal lesions or abnormal FDG uptake.



Name : Amal Reyad Abdel Khalek Hussein

Date : 10-02-2026 10:56

Branch : Portable

Medical No. : GRI-77262

Ref. Physician Prof : CT CAP+C

Ins. Company :

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

➤ Known case of right BCS . Compared to the last study 18/11/2025, The current revealed:

1. Chest:

- **No appreciable time interval changes** regarding to the size of the few right axillary lymph nodes became prominent the largest measures about 1x0.5 cm.
- Rather stationary course regarding the previously seen few tiny bilateral scattered pulmonary nodules largest measuring 3.5 mm in diameter at the left lower lobe.
- **No appreciable time interval changes regarding the number of the previously seen few bilateral scattered faint ground glass densities.**
- **Still noted unchanged** bilateral apical pleuropulmonary scarring.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is of average size *with regressive course regarding the previously seen few bilobar* hypodense focal lesions, The largest at segment VIII measuring currently about 6 mm in diameter in today's study.
- Patent homogenous portal vein and its main tributaries
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Regressive course mounting to complete resolution regarding the previously seen right ovarian cyst.
- Normal appearance of the other pelvic organs.
- Regressive course mounting to complete resolution regarding the previously seen mild pelvic collection.

3. The scanned skeleton:

Name : Amal Reyad abdel khalek hussei

Branch : Portable

Ref.Physician Prof : CT CAP+C

Date : 10-02-2026 10:56

Medical No. : GRI-77262

Ins.Company :

- No appreciable time interval changes regarding the previously seen few vertebral and left iliac wing lytic as well as the D5 sclerotic focus.
- Still seen 1st degree lytic spondylolisthesis of L4.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Reported case of right BCS SHOWING

- Regressive course regarding the:
 - The size and number of the previously seen few bilobar hypodense hepatic focal lesions.
 - The previously seen right ovarian cyst no longer seen in today's study.
 - The previously seen mild pelvic collection.
- Stationary course regarding the rest of the study.

Dr. Sherif Wagdy

Dr. Sherif Wagdy

Name : Amaal ryad abd elkhaliq hussin

Branch : Portable

Ref. Physician Prof : brain +c

Date : 10-02-2026 11:20

Medical No. : GRI-

260210111719STD131221107523748902

Ins. Company :

MRI OF THE BRAIN

Technique :

Axial T1, T2 ,FLAIR, DWI& ADC

Coronal T2

Sagittal T2

Postcontrast T1

Findings: compared to last study dated 21-6-2025 ;

- Marked regressive course as regard the size and shape of the previously noted multiple bilateral cerebral SOL seen at both frontal , parietal basal ganglia and , both thalami and right subependymal and left cerebellar region , the lesion currently seen as foci and small areas of altered MR signal eliciting low T1 and bright T2 and FLAIR W1 signal with contrast uptake, some of them show minimal surrounding edema, no significant mass effect ,the largest one is seen at right lateral ventricle sub endymal location measuring about 6mm compared to larest one 14 mm in the las study with regression of the previously noted marked perifocal edema .
- Normal size and configuration of the supra and infra tentorial ventricular system
- No midline shift or deformity
- Normal cortical sulci
- No cerebello pontine angle masses,
- Partial empty sella.
- Prominent peri-optic CSF spaces bilaterally.

OPINION:

- *Marked regressive course of the previously noted cerebral and cerebellar intra axila SOLs compared to last study , for follow up and clinical correlation.*